BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety	Validated RET mutation test; full blood count, kidney and liver tests, potassium, calcium, magnesium, thyroid-stimulating hormone, electrocardiogram (ECG) and blood pressure. Baseline calcitonin is additionally advised.	Blood count, kidney and liver tests at weeks 1, 3, 6 and 12. ECG, potassium, calcium, magnesium and thyroid-stimulating hormone at weeks 1, 3, 6 and 12, then every 3 months for at least a year. Repeat this schedule after a QT-related reduction or interruption longer than 2 weeks. Continue checks after stopping when clinically indicated; monitor blood pressure.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines	Agree the treatment aim, dose and complete medicines list.	Take at the same time daily, with or without food. If a dose is missed, take it when remembered unless less than 12 hours remain before the next dose; then skip it. Never double a dose. If swallowing is difficult, disperse the whole tablet without crushing in half a glass of still drinking water, stir for about 10 minutes and drink immediately; rinse the glass with another half-glass and drink. Use no other liquid. Pharmacy must teach feeding-tube administration if needed.	Keep a current dose and symptom record.	Carry the manufacturer's patient alert card and receive one with every prescription. Report fainting or palpitations urgently. Check every new medicine for heart-rhythm interactions. Diarrhoea can disturb blood salts and increase rhythm risk. Protect skin from sunlight.	Keep follow-up and recovery checks even after the last dose.

Check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Cancer-specific and specialist tests	RET is the treatment-selection test. Calcitonin is measured at baseline and later when clinically indicated to help assess medullary thyroid cancer. Thyroid-stimulating hormone measures hormone replacement needs.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00242 v6: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Validated RET mutation test; full blood count, kidney and liver tests, potassium, calcium, magnesium, thyroid-stimulating hormone, ECG and blood pressure. Baseline calcitonin is additionally advised.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Blood count, kidney and liver tests at weeks 1, 3, 6 and 12. ECG, potassium, calcium, magnesium and thyroid-stimulating hormone at weeks 1, 3, 6 and 12, then every 3 months for at least a year. Repeat this schedule after a QT-related reduction or interruption longer than 2 weeks. Continue checks after stopping when clinically indicated; monitor blood pressure.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

RET is the treatment-selection test. Calcitonin is measured at baseline and later when clinically indicated to help assess medullary thyroid cancer. Thyroid-stimulating hormone measures hormone replacement needs.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Vandetanib: RET-positive medullary thyroid cancer: treatment and checks

RET mutation positive, unresectable locally advanced or metastatic medullary thyroid cancer.

Vandetanib 300 mg once daily, continuously, until progression or unacceptable side effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Validated RET mutation test; full blood count, kidney and liver tests, potassium, calcium, magnesium, thyroid-stimulating hormone, ECG and blood pressure. Baseline calcitonin is additionally advised.	Blood count, kidney and liver tests at weeks 1, 3, 6 and 12. ECG, potassium, calcium, magnesium and thyroid-stimulating hormone at weeks 1, 3, 6 and 12, then every 3 months for at least a year. Repeat this schedule after a QT-related reduction or interruption longer than 2 weeks. Continue checks after stopping when clinically indicated; monitor blood pressure.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take at the same time daily, with or without food. If a dose is missed, take it when remembered unless less than 12 hours remain before the next dose; then skip it. Never double a dose. If swallowing is difficult, disperse the whole tablet without crushing in half a glass of still drinking water, stir for about 10 minutes and drink immediately; rinse the glass with another half-glass and drink. Use no other liquid. Pharmacy must teach feeding-tube administration if needed.	Keep a current dose and symptom record.	Carry the manufacturer's patient alert card and receive one with every prescription. Report fainting or palpitations urgently. Check every new medicine for heart-rhythm interactions. Diarrhoea can disturb blood salts and increase rhythm risk. Protect skin from sunlight.	Keep follow-up and recovery checks even after the last dose.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Cancer-specific and specialist tests NCCP	RET is the treatment-selection test. Calcitonin is measured at baseline and later when clinically indicated to help assess medullary thyroid cancer. Thyroid-stimulating hormone measures hormone replacement needs.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

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Treatment and review

Once daily, continuously

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / act: Blood count, kidney and liver tests at weeks 1, 3, 6 and 12. ECG, potassium, calcium, magnesium and thyroid-stimulating hormone at weeks 1, 3, 6 and 12, then every 3 months for at least a year. Repeat this schedule after a QT-related reduction or interruption longer than 2 weeks. Continue checks after stopping when clinically indicated; monitor blood pressure.

Sources: NCCP

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.50